BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. National Cancer Control Programme (NCCP) remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan. Metastatic T-DM1 continues according to benefit and tolerance; the early-breast 14-cycle course does not apply.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment. Review cardiac follow-up when treatment changes.
Blood tests	Baseline full blood count (FBC), renal and liver profile	Before each cycle	Repeat before this cycle	Before every cycle	Every cycle
electrocardiogram (ECG) / rhythm	ECG if clinically indicated; baseline echo / MUGA required.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Record the disease sites and comparison imaging	Assess symptoms at every visit	Scan if due or indicated	Response review on the individual plan	Metastatic response scans commonly every 2–4 months; markers supplement the full assessment.
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	Assess bone-health need from the individual treatment and fracture risk	Not an automatic T-DM1 test	Timing depends on risk and results	Separate from echo monitoring	Any endocrine treatment or bone protection requires its own oncology plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00206 v5 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Kadcyla product information · metastatic toxicity table

THE WHOLE TREATMENT JOURNEY

Trastuzumab emtansine (T-DM1) · metastatic: the whole pathway

Previously treated HER2-positive unresectable locally advanced or metastatic breast cancer

T-DM1 3.6 mg/kg intravenous (IV) every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00206	Confirm setting, prior doses and formulation	T-DM1 IV every 3 weeks	Review benefit, symptoms and heart function	Continue while beneficial and tolerated	Review cancer control and tolerance; agree continuation or a change.
Surgery METASTATIC	Local procedures only if indicated	No automatic curative surgical sequence	Coordinate procedures with oncology	Symptom / site-specific decisions	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Timing depends on the clinical need	Agree any hold / continuation with radiation oncology	radiotherapy (RT) is site / symptom directed. Agree any treatment hold with both teams.	Radiation effects and surveillance follow the local plan
Blood tests NCCP 00206	Baseline FBC, renal and liver tests	Before every cycle	Before every cycle, including around C5	Before every cycle	Review abnormalities and the next treatment plan
Heart checks NCCP 00206 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Follow-up depends on cardiac findings and the next treatment plan.
Scans / markers EARLY · METASTATIC · MARKERS	Define baseline sites and scan purpose	Review symptoms at each visit	Response imaging when due; exact timing agreed for the known disease sites.	Continue the individual metastatic response-assessment plan.	Response scans commonly every 2–4 months; individualise. Markers supplement clinical review and imaging.

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Endocrine / bone health EARLY · BONE	Check hormone receptors, previous treatments and bone-health needs	Any endocrine treatment requires a separate oncology plan	DEXA only if indicated by endocrine treatment or fracture risk	Do not add another anticancer medicine from this component card	Continue the individual bone and supportive-care plan

What each check is for

1

Blood tests

FBC, renal and liver profile before every cycle

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

Sources: NCCP 00206

2

Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

Sources: NCCP 00206 · SMPC

3

Cancer imaging and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

Sources: EARLY · METASTATIC · MARKERS

4

Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00206

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-21 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.14